OFFICE OF THE UNIVERSITY REGISTRAR & ACADEMIC SENATE

ACADEMIC QUALITY ASSURANCE, EXAMINATIONS & STUDENT WELFARE GOVERNANCE FRAMEWORK

EXAMINATION AND MEDICAL CONSIDERATION REGULATIONS

Document Code:	**POL-ACAD-EX-01**	**Version & Status:**	Version 4.0 (2025 Mandatory Senate Revision)
Effective Date:	01 January 2025	**Supersedes:**	Section 5.1 of Legacy Student Handbook (HBK-ACAD-PR-05 v1.8, 2023)
Approving Body:	University Senate & Board of Examinations	**Linked Policies:**	POL-ACAD-AT-02, POL-ACAD-CW-03, POL-MED-WF-04, HBK-ACAD-PR-05

SECTION 1: REGULATORY PURPOSE, SCOPE, AND STATUTORY DEFINITIONS

Clause 1.1 (Purpose and Institutional Scope): This policy establishes the binding regulatory framework governing undergraduate and postgraduate taught students who are prevented from attending scheduled end-of-semester written examinations, laboratory practical examinations, Objective Structured Clinical/Practical Examinations (OSCE/OSPE), or formal viva voce assessments due to acute, unforeseen medical circumstances. All faculties, examination boards, and administrative registries must enforce the timelines and evidentiary standards specified herein.

Clause 1.2 (Supersession of Legacy Provisions): Effective 01 January 2025, this regulation (POL-ACAD-EX-01 v4.0) explicitly supersedes all conflicting examination deferral clauses contained in the 2023 Student Handbook (HBK-ACAD-PR-05 v1.8). Specifically, the legacy 7-calendar-day submission window is hereby revoked and replaced by the strict **5-working-day** statutory deadline established in Clause 3.2.

KEY REGULATORY DEFINITIONS (PART A)

- **Extenuating Circumstances (EC):** Sudden, unforeseen, acute medical events or hospitalization beyond a student's control that demonstrably incapacitate examination performance.
- **Deferred Examination (Grade 'I-Med'):** An authorized postponement of a scheduled examination resulting in an unpenalized First-Sit attempt during the Supplementary Examination Window.
- **Fit-to-Sit Principle:** The legal presumption that any student who enters an examination venue and signs the attendance sheet declares themselves medically fit to sit the assessment.

KEY REGULATORY DEFINITIONS (PART B)

- **Form EC-01:** Official Application for Examination Medical Consideration, submitted digitally and in physical copy to the Examinations Division.
- **Form INV-09:** Chief Invigilator's Acute Mid-Examination Illness Incident Log, mandatory for students taken ill inside an examination hall.
- **Form MED-04-CLR:** University Chief Medical Officer (CMO) Verification Clearance Slip required under Policy POL-MED-WF-04 before Examinations Board approval.

Table 1.1: Classification of Examination Medical Scenarios, Grade Codes, and Academic Remedies

Scenario Code	Clinical / Examination Circumstance	Transcript Code	Academic Outcome & Mark Cap	Required Primary Forms
EX-MED-A	Total absence from final exam due to inpatient hospitalization (>= 24 hours)	I-Med (Deferred)	Uncapped First-Sit (100% max mark) in Supplementary Exam Window	Form EC-01 + Hospital Discharge Ticket + Form MED-04-CLR
EX-MED-B	Total absence from final exam due to acute outpatient medical incapacitation	I-Med (Deferred)	Uncapped First-Sit (100% max mark) subject to SLMC & CMO verification	Form EC-01 + SLMC Medical Certificate + Form MED-04-CLR
EX-MED-C	Acute incapacitation mid-examination (student evacuated before completing >50% of time)	I-Med (Voided Sit)	Original script voided; Uncapped First-Sit in Supplementary Window	Form INV-09 + Campus Medical Officer Report + Form EC-01
EX-MED-D	Student completes examination and claims illness retrospectively after sitting	Standard Grade	Claim Rejected under Fit-to-Sit Rule (Clause 2.1); original mark stands	Not Eligible (unless acute collapse logged via Form INV-09)
EX-MED-E	Absence without notification or submission after 5 working days (unexcused)	ABS / 0% (Fail)	Zero mark recorded; Repeat attempt capped at Pass Mark (40%)	Late Appeal Form APP-EX-02 (only if incapacitated in ICU)

SECTION 2: THE 'FIT-TO-SIT' DOCTRINE AND MID-EXAMINATION INCAPACITATION

Clause 2.1 (Strict Enforcement of the Fit-to-Sit Declaration): The University operates a strict 'Fit-to-Sit' regulatory regime. By entering an examination room, receiving the question paper, and signing the attendance register, a student formally declares that they are medically and psychologically fit to undertake the examination. Retrospective medical claims submitted after a student has completed an examination—or after provisional marks have been published—shall be summarily rejected by theExaminations Committee.

Clause 2.2 (Acute Illness Occurring During an Examination Session): Where a student experiences a sudden, severe medical emergency after an examination has commenced (e.g., acute syncope, severe migraine aura, asthma attack, or high fever), the student must comply with all three mandatory steps below to void the sitting:

Sub-clause 2.2.1 (a): Immediately raise a hand and notify the Chief Invigilator prior to leaving the examination hall. Leaving the venue without invigilator recording invalidates any subsequent medical claim.

Sub-clause 2.2.1 (b): Ensure the Chief Invigilator records the exact time of departure and clinical symptoms on the *Acute Mid-Examination Illness Incident Log (Form INV-09)* and seals the partially completed answer script separately.

Sub-clause 2.2.1 (c): Report directly under escort to the University Medical Center (UMC) within **two (2) hours** of leaving the examination hall for an immediate clinical evaluation by the Campus Medical Officer (CMO), or be transported directly by ambulance to a government/registered hospital.

MANDATORY REGULATORY WARNING — FIT-TO-SIT RULE (CLAUSE 2.1)
Students who feel unwell on the morning of an examination MUST NOT enter the examination hall hoping to 'try the paper first.' Once a student remains in the hall past the first 30 minutes without logging an acute emergency on Form INV-09, the examination sitting is legally binding and cannot be annulled by a medical certificate obtained later that day.

SECTION 3: MANDATORY TIMELINES AND APPLICATION PROCEDURE FOR MISSED EXAMINATIONS

Clause 3.1 (Stage 1 — Initial Digital Notification within 48 Hours): A student who misses a scheduled final examination due to hospitalization or acute medical incapacity must log into the *Student Academic Portal (Examinations -> Extenuating Circumstances)* and submit an Initial Absence Notification within **forty-eight (48) hours** of the scheduled examination start time. Where a student is hospitalized in an Intensive Care Unit (ICU) or surgical ward and physically unable to access the portal, a parent, legal guardian, or next-of-kin may notify the Office of the Registrar via official email (`exams.welfare@university.edu`) within the same 48-hour window.

Clause 3.2 (Stage 2 — Formal Application and Medical Dossier within 5 Working Days): Following initial notification, the student must submit the completed *Application for Examination Medical Consideration (Form EC-01)* together with verified original medical documentation to the Examinations Division within **five (5) working days** from the date of the

missed examination. **Hospitalization Extension Exception:** If a student remains continuously hospitalized as an inpatient for more than five working days (for example, a 10-day hospitalization), the 5-working-day submission clock commences on the **date of official hospital discharge** recorded on the Hospital Discharge Summary.

Clause 3.3 (Mandatory Two-Step Medical Verification under POL-MED-WF-04): The Examinations Division will not accept any medical certificate directly at the Examinations Counter unless it has first been inspected and stamped by the University Chief Medical Officer (CMO) at the University Medical Center (Building B, Ground Floor) and is accompanied by an issued *Official Medical Clearance Slip (Form MED-04-CLR)* as mandated by Policy POL-MED-WF-04 Section 4.2.

Table 3.1: Statutory Deadlines and Submission Workflow for Examination Medical Consideration

Workflow Stage	Action Required by Student / Representative	Statutory Deadline (2025 Rule)	Receiving Office / System	Consequence of Non-Compliance
Stage 1: Alert	Submit Online Absence Alert + upload preliminary scan/photo of admission note	Within 48 hours of missed examination start time	Student Academic Portal (EC Module) or Registrar Email	Application flagged as Unnotified; requires Late Explanation Memo
Stage 2: CMO Vetting	Present original SLMC Medical Certificate / Hospital Discharge Ticket for CMO verification	Within 5 working days of exam (or within 5 working days of hospital discharge)	University Medical Center (UMC), Building B, Ground Floor	Examinations Counter will refuse unverified medical certificates
Stage 3: Formal Filing	Submit Form EC-01 + Form MED-04-CLR + Original Medical Dossier + Exam Timetable copy	Within 5 working days of exam (or within 5 working days of hospital discharge)	Examinations Division Counter (Admin Block, Level 2, Room 204)	Automatic rejection; grade recorded as ABS (0% Fail)
Stage 4: Board Review	Faculty EC Panel & Senate Examinations Board ratifies 'I-Med' Deferred status	Within 10 working days of dossier submission	Senate Examinations Committee (Notification via Portal)	Student receives official Deferred Exam Entry Permit

SECTION 4: VALID VS. INVALID GROUNDS FOR EXAMINATION CONSIDERATION

Clause 4.1 (Admissible Medical Evidence): To qualify for an uncapped deferred examination (`I-Med`), medical evidence must demonstrate an acute, incapacitating medical condition assessed contemporaneously by an SLMC-registered medical practitioner or government hospital specialist (see POL-MED-WF-04 Section 2). For inpatient hospitalizations (such as a 10-day admission for dengue hemorrhagic fever, appendectomy, pneumonia, or acute trauma), the student must provide both the *Official Inpatient Admission & Discharge Ticket* and the *Ward Consultant's Clinical Summary* covering all missed examination dates.

Clause 4.2 (Excluded and Invalid Grounds): The Examinations Committee shall reject applications based on minor, self-limiting illnesses or non-medical administrative negligence, as itemized in Table 4.1 below.

Table 4.1: Admissible vs. Inadmissible Grounds for Medical Examination Deferral

Evidence Category	Specific Condition / Circumstance	Eligibility Status	Mandatory Evidentiary Standard / Rationale
Inpatient Hospitalization	Hospital admission >= 24 hours (e.g., 10-day hospitalization for acute illness/surgery)	ELIGIBLE (Priority Approval)	Hospital Discharge Summary + Ward Consultant Stamp + Form MED-04-CLR
Acute Outpatient Illness	Contagious infection (chickenpox, severe influenza), acute fracture, high-grade fever	ELIGIBLE (Subject to CMO Review)	SLMC Registered Physician Certificate stating 'Unfit for Exam' + prescriptions/lab reports
Acute Mental Health Crisis	Severe panic disorder crisis, psychiatric emergency, or acute bereavement trauma	ELIGIBLE (Confidential Track)	Psychiatrist Certificate or University Senior Counselor Endorsement (POL-MED-WF-04 Sec 5)
Chronic Condition Flare-up	Acute exacerbation of documented chronic illness (e.g., epilepsy, severe Crohn's disease)	ELIGIBLE	Specialist Consultant letter confirming acute flare-up on exam date
Minor / Self-Limiting Ailment	Mild common cold, mild headache, hay fever, normal examination anxiety, sleeplessness	INELIGIBLE (Rejected)	Does not constitute clinical incapacitation under Senate regulations
Retrospective / Backdated Note	Doctor consulted days after the missed exam stating 'student told me they were ill'	INELIGIBLE (Rejected)	Consultation must be contemporaneous with the illness (POL-MED-WF-04 Clause 2.3)
Administrative Negligence	Misreading examination timetable, oversleeping, traffic congestion, lost student ID card	INELIGIBLE (Rejected)	Non-medical student responsibility; zero mark (0%) recorded

SECTION 5: DEFERRED EXAMINATION SCHEDULING, REPEAT ABSENCES, AND APPEALS

Clause 5.1 (Administration of Deferred Examinations): Students granted an `I-Med` grade shall sit the deferred examination during the official *Supplementary & Deferred Examination Window*, normally scheduled **four (4) to six (6) weeks** after the publication of main semester results. An approved `I-Med` sitting is treated as a **First-Sit attempt**: the student is eligible for the full mark range (0%–100%), no deferred examination fee is charged, and the transcript reflects a normal first-attempt grade once completed.

Clause 5.2 (Medical Absence from a Scheduled Deferred Examination): The University does not organize individual ad-hoc third examination sittings within the same semester. If a student granted an `I-Med` deferral is medically incapacitated a second time during the Supplementary Examination Window, the student must submit a fresh `Form EC-01` within 5 working days. Upon verification, the student's `I-Med` status is carried forward to the next regular semester offering of the module, or—if total pending credits exceed 30 credits—the student is placed on *Temporary Medical Intercalation* under `HBK-ACAD-PR-05` Section 3.3.

Clause 5.3 (Practical, Laboratory, and OSCE Examinations): Where a missed final examination involves a laboratory bench examination or clinical OSCE, the Head of Department may authorize either a practical make-up session during the Supplementary Window or, where laboratory reconstruction is impracticable, an rigorous Oral Examination (Viva Voce) and Practical Portfolio Assessment of equivalent learning outcome weight.

Clause 5.4 (Examination Medical Appeals Procedure): A student whose `Form EC-01` application is rejected by the Faculty EC Panel may lodge a formal appeal to the *Senate Examinations Appeals Board* using **Form APP-EX-02** within **ten (10) working days** of receiving the rejection notice. An appeal is admissible only on two grounds: (a) availability of new hospital or diagnostic evidence that could not reasonably have been obtained within the original 5-working-day window, or (b) demonstrable procedural irregularity in the processing of the original application.

Table 5.1: Deferred Examination Grading, Progression Impact, and Fee Structure Matrix

Attempt Type	Transcript Grade Symbol	Mark Cap Policy	Examination Fee	Impact on Academic Progression (HBK-ACAD-PR-05)
Approved 1st Medical Deferral	I-Med -> Actual Grade (A+ to F)	Uncapped (100% Max)	Rs. 0 (100% Fee Waiver)	Counts as Pending Non-Penalized Credits; allows provisional progression if <= 30 credits
Fail in Deferred Exam (1st Sit)	F (Resit Eligible)	Resit Capped at 40% (C)	Standard Resit Fee applies	Student retains standard Second-Sit (Resit) opportunity in next cycle
Approved 2nd Medical Deferral	I-Med-Ext (Extended)	Uncapped (100% Max)	Rs. 0 (Fee Waiver)	Sits with next cohort; mandatory Intercalation if total pending credits > 30
Unapproved Examination Absence	ABS (0% Marks)	Resit Capped at 40% (C)	Full Repeat Fee + Fine	Counts as 0 earned credits; may trigger Academic Probation

POLICY CONFLICT & SUPERSESSION NOTICE (2025 vs. 2023 HANDBOOK)
Note for Academic Advisors & Automated Compliance Systems: Legacy Clause 5.1 of the 2023 Student Handbook (HBK-ACAD-PR-05 v1.8) states that medical certificates for missed examinations may be submitted within '7 calendar days.' That rule is OBSOLETE. Under POL-ACAD-EX-01 v4.0 (2025) Clause 3.2, the binding statutory deadline is 5 WORKING DAYS from the exam date (or 5 working days from official hospital discharge for inpatient stays).